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PRE-OPERATIVE ASSESSMENT & PREPARATION

Eating & Drink Before Surgery

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Key Points

- **Avoid unnecessary thirst and dehydration in patients prior to surgery.**
- **Actively encourage patients to take oral fluids.**
- **Clear fluids include: water, squash, non-pulp fruit juice, carbohydrate (Preop) drinks, tea/coffee or Fortijuice.**
- **Do not postpone an operation if patients are found to be chewing gum or sucking a boiled sweet immediately before the induction of anaesthesia.**
- **Caution: Patients on GLP-1 agonists may have delayed gastric emptying.**

Cautions & Contraindications

This guide excludes those patients who are 'nil by mouth' for reasons other than awaiting surgery. e.g. bowel obstruction, inability to swallow.

Patients on GLP-1 agonist e.g. Semaglutide may have delayed gastric emptying for up to 7 days after the last dose. Anaesthetists should evaluate the most appropriate strategy to mitigate the potential risk of aspiration during induction, maintenance and emergence phases.

Considerations

- It is the joint responsibility of staff on the ward or admissions lounge and theatres to ensure that patients remain adequately hydrated before surgery.
- Unless contraindicated**, All patients** awaiting theatre should be provided with a jug of water and encouraged to drink.
- The term **'Nil by Mouth'** should be only used for patients who are strictly nil by mouth for reasons other than awaiting surgery.
- If oral fluids are contraindicated, then initiate iv fluid maintenance with a balanced salt solution.
- Plan to do younger children as early as possible on the list to avoid long fasting periods.
- Patients undergoing emergency surgery should also follow these fasting guidelines, as appropriate.
- Further guidance may be given by the anaesthetist.

- Wards should be informed of any changes to theatre list order or delays in sending for patients.
- **Chewing gum** and **sucking sweets** should ideally not be used less than 6 hours pre-operatively. However, patients should not have their operation cancelled or delayed just because they are chewing gum or sucking a boiled sweet immediately prior to induction of anaesthesia.

Adults

Default Rule:

Provide each patient with a jug & cup of still water. Advise them to drink water as they wish. If you do not know the estimated time of Surgery, then encourage drinking of water only.

When estimated Time of Surgery is known:

6 hours:

Patients should have no food (**including** milk-based drinks, fortisips & sweets) for a minimum period of six hours before surgery.

2 hours: Encourage 'Clear' Fluids up to two hours before sending.

Clear' fluids include water, squash, non-pulp fruit juice, tea/coffee, carbohydrate (Preop) drinks or Fortijuice) up to 2 hours before surgery. No bits, fizz or milk!

Within 2 hours: Water Only

Children > 6 months

Default Rule:

Provide each patient with a jug & cup of still water. Advise them to drink water as they wish. If you do not know the estimated time of Surgery, then encourage drinking of water only.

When estimated Time of Surgery is known:

6 hours:

Children should have no food or milk (including formula) for a minimum period of six hours before surgery.

1 Hour: Encourage 'Clear' Fluids up to one hour before sending.

'Clear' fluids include water, clear fruit juice or squash/cordial, ready diluted drinks and non-carbonated sports drinks. No bits, fizz or milk!

Neonates and babies on breast and formula milk: 6-3-1 Rule

Clear fluids up to and at 1 hours before the induction of anaesthesia.

Breast milk up to and at 3 hours before the induction of anaesthesia.

Formula milk up to and at 6 hours before the induction of anaesthesia.

Antacids

Patients should continue their regular antacids on the day of surgery. The anaesthetic team should consider further interventions for patients at higher risk of regurgitation and aspiration (eg ranitidine or PPI.)

Below are the instructions that should be included in TCI letters sent to patients. If you find a patient has not followed the correct advice or claims to have been given different advice, ask the surgeon's secretary for a copy of the advice they send and check it is the same as below.

Procedures under Local Anaesthesia (excluding Endoscopies)

Please bring both your medication list and all you current medications with you. You can have a light breakfast if your admission is in the morning or a light lunch if your admission is in the afternoon. Make sure you continue to drink during the day.

Instructions for Adult Patients undergoing Regional or General Anaesthesia

For patients on morning / all day lists:

You may eat as normal on the day before surgery. Do not eat anything nor drink milk after midnight. Make sure you keep hydrated. You may drink cordial, black tea/coffee, non-pulp fruit juice up until 7am, but only water after this time. Please bring both your medication list and all you current medications with you. Take you morning medications as normal, as per instructions given to you in the Pre-Operative Assessment clinic.

For patients on an afternoon list:

You may eat as normal on the day before surgery. On the day of surgery, you may have a light breakfast (e.g. 1 slice of toast or bowl of cereal) before 7:00am. Make sure you keep hydrated. You may drink cordial, black tea/coffee, non-pulp fruit juice up until 11am, but only water after this time. Please bring both your medication list and all you current medications with you. Take you morning medications as normal, as per instructions given to you in the Pre-Operative Assessment clinic.

Instructions for Paediatric Patients

For patients on morning / all day lists:

Please do not let your child eat anything nor drink milk after midnight. Make sure they keep hydrated. They may drink cordial or non-pulp fruit juice up until 8am, but only water after this time. Please bring their medication list and all their current medications with you.

For patients on an afternoon list:

Please ensure your child is given a light breakfast, eaten before 7am. Make sure they keep hydrated. They may drink cordial or non-pulp fruit juice up until 12pm, but only water after this time. Please bring their medication list and all their current medications with you.

References

1. AAGBI safety guideline 2015: Pre-operative assessment and patient preparation
2. Preoperative Fasting Guidelines: UpToDate October 2017
3. BJA Education, 17 (10:346-350 (2017)
4. APA Consensus guidance update April 2018